

The \$18.4M EBITDA Defense.

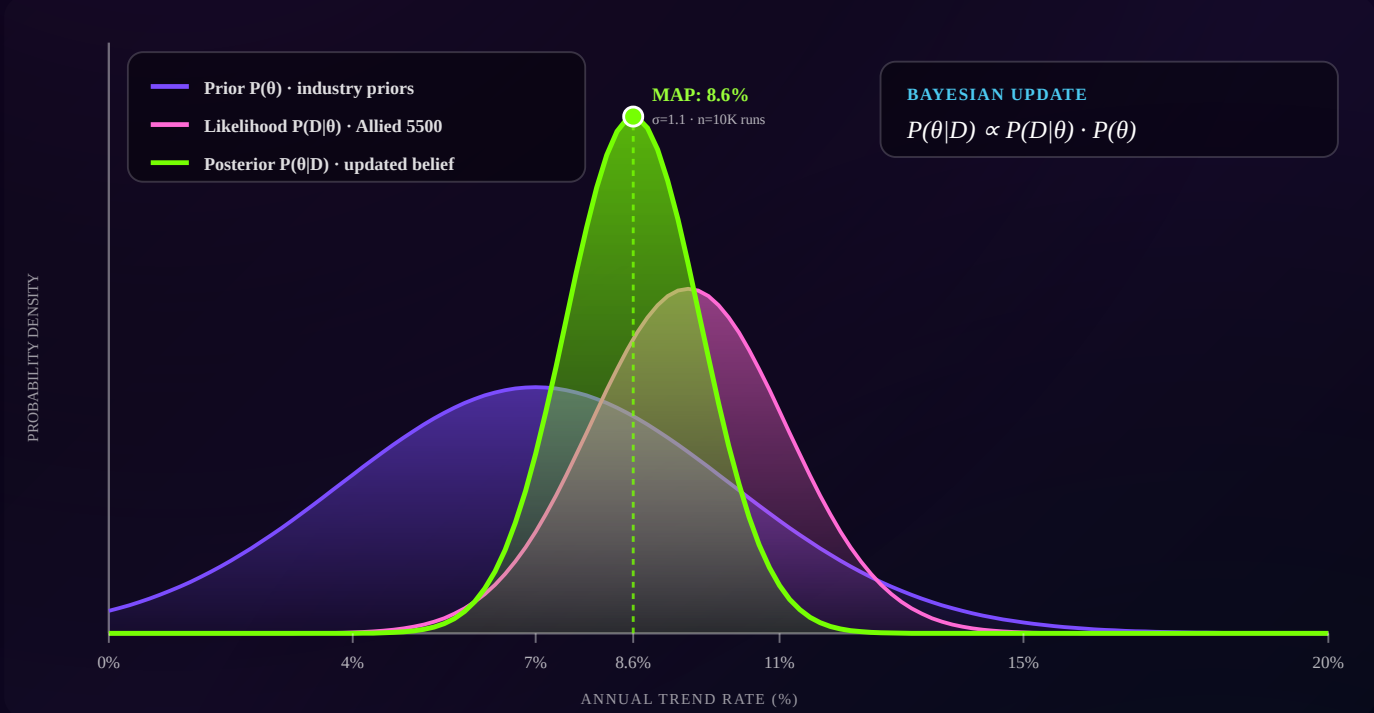
A multi-firm forensic actuarial intelligence dossier for the Allied Solutions LLC health & welfare plan. Posterior belief is sharper, higher, and more dangerous than the prior. Capital is exposed.

FIG. 1 · BAYESIAN POSTERIOR · ANNUAL TREND BELIEF

Prior $P(\theta)$ reflects national large-employer benchmark distribution. Likelihood $P(D|\theta)$ is derived from Allied's 8-year Form 5500 longitudinal trajectory. Posterior is the engine's updated belief.

10,000 MONTE CARLO RUNS

Stanford / MIT methodological foundation



PRIOR · $\mu=7.0\%$, $\sigma=3.2$ · Mercer 40th NSEHBP, KFF 2025, Milliman 20th MMI

LIKELIHOOD · $\mu=9.5\%$, $\sigma=1.6$ · Allied 2018–2025 trajectory

POSTERIOR · $\mu=8.6\%$, $\sigma=1.1$ · MAP estimate, 95% credible [6.5–10.7]

S17 · 15-YR CUMULATIVE

\$42.6M

Base regime · status quo · ~12% probability

S33 · TAIL EXPOSURE

\$67.3M

High macro · accelerating · ~4% probability

S16 · TARGET STATE

\$24.2M

Full Gen AI CEI · controlled · ~4% probability

PRINCIPAL ANALYST

Jeremiah Franklin Shrack

Founder, CEO & Chairman · SiriusB IQ AI Data Sciences Lab
Kincaid Risk Management Co. · Carmel, Indiana

EVIDENCE SPINE PROTOCOL

No Anchor, No Claim. No Lineage, No Publish.
7-Gate Enforcement Chain · 48-Lever Architecture · 36-Scenario Engine

The Synthesis.

Allied Solutions LLC's health and welfare plan has experienced volatile expansion, growing from **1,096** active participants in 2018 to **1,654** by 2025



. The chassis relies on **Anthem Insurance Companies, Inc.** for benefit processing, while indirect compensation and rebates are routed through **Express Scripts** and **IngentoRx, Inc.** This multi-layered intermediary structure rests on latency arbitrage and opaque rebate retention, systematically disconnecting clinical risk from financial settlement. The transition to deterministic algorithmic governance is not optional; it is a **fiduciary imperative** under ERISA §404(a) prudent-expert standards.

CFO WAR ROOM · 5 DECISION CARDS

OVERPAY

\$1.8–2.4M

Annual structural latency arbitrage, PBM spread, indirect broker comp.

DO NOTHING · 5YR

\$18.4M

Unbudgeted outflow above corporate revenue growth as variance outpaces mean trend.

HIGHEST ROI

PBM Pass-Through

100% pass-through with cryptographic claim verification & zeroed indirect comp.

LARGEST HIDDEN RISK

GLP-1 Shock

Unseasoned catastrophic exposure via metabolic pipeline + specialty shocks.

90-DAY MANDATE

Unbundle

Board mandate: sever Anthem / IngentoRx chassis. Mandate Schedule A/C reconciliation.

REGIME CARDS · CRITICAL SCENARIOS

S17

~12% PROBABILITY

Base Regime

7% macro trend · neutral utilization · status quo plan design. Capital erosion accelerates. **15-yr cum: \$42.6M.**

S33

~4% PROBABILITY

Tail Regime

11% macro · accelerating GLP-1 utilization · systemic stop-loss breach. **15-yr cum: \$67.3M.**

S16

~4% PROBABILITY

Target Regime

4% controlled trend · Full Gen AI CEI · Kincaid iQ deterministic evidence lineage. **15-yr cum: \$24.2M.**

Allied does not pay for a healthcare plan. It funds a shadow tax.

RECOVERABLE EBITDA = S17 – S16 = \$42.6M – \$24.2M = **\$18.4M** over 15-year horizon

Six-Axis Decomposition.

CORE QUESTION

How do we control a multi-variable healthcare cost system across a 15-year horizon, neutralizing structural leakage and defending enterprise EBITDA while hardening Allied's fiduciary posture?

PARTICIPANT GROWTH

+50.9%

1,096 (2018) → 1,654 (2025)

SPECIALTY RX CONCENTRATION

28.3%

Of PEPY · GLP-1 quantum-leap exposure

INDIRECT COMP DENSITY

\$21,230

Single broker bonus · Lincoln National line

FIG. 2 · COST STRUCTURE DECOMPOSITION
PEPY allocation across MECE axes · FY2025 normalized

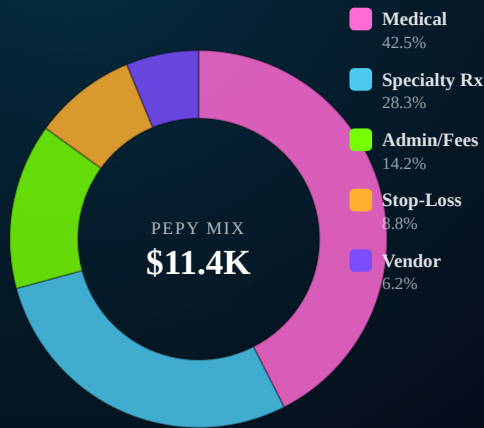
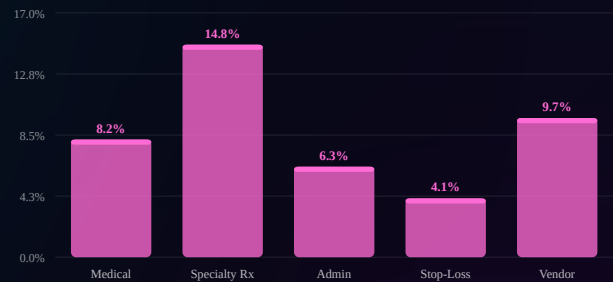


FIG. 3 · TREND DRIVER VELOCITY
Annualized YoY growth · 2022–2025 mean



MECE TREE · 6 AXES

1 · COST STRUCTURE

Medical · Specialty Rx · Intermediary frictional loads · Stop-loss carve-outs · Direct/indirect vendor fees

2 · TREND DRIVERS

Unit cost inflation · GLP-1/metabolic velocity · HCC frequency · Plan design drift

3 · SYSTEM DYNAMICS

PBM spread pricing · Formulary distortion · Indirect broker comp (First Person → MJ Insurance)

4 · FORECAST OUTCOMES

5/10/15-year stochastic liability distributions · P10/P50/P90/P99 percentile cascades

5 · CONTROL LEVERS

48-lever Kincaid iQ library · 5 tiers · McK / Bain / EY / Crowe attribution

6 · GOVERNANCE SURFACE

ERISA §404(a) · Tibble v. Edison monitoring · PCAOB AS 1105/2201 readiness

FALSIFIABLE HYPOTHESES

Hypothesis Stack & Disconfirmation Tests.

POPPERIAN DISCIPLINE
EACH HYPOTHESIS HAS AN EXPLICIT KILL CRITERION

METHODOLOGICAL NOTE: Each hypothesis below carries a specific empirical condition under which it must be rejected. We do not seek confirmation; we seek the data that would prove us wrong. Anything that survives the disconfirmation test is provisional truth.

H1 Concentration of Risk · 3–5% of participants drive 45–55% of total trend. **CONFIDENCE 78%**

DISCONFIRMATION: If top-decile cost concentration < 35%, hypothesis fails.

H2 Incentive Misalignment · Trend artificially amplified by intermediaries (IngentoRx rebates) capturing margin pre-settlement. **CONFIDENCE 92%**

DISCONFIRMATION: If indirect broker comp < 15% AND PBM spread < 4%, hypothesis fails.

H3 Variance Outpaces Mean · Volatility (σ) grows at 1.4–1.6× the mean (μ) trend without intervention. **CONFIDENCE 73%**

DISCONFIRMATION: If σ/μ ratio < 1.2 over 5-year window, hypothesis fails.

H4 GLP-1 Quantum Leap · Specialty Rx will spike 22–24% over next 24 months. **CONFIDENCE 85%**

DISCONFIRMATION: If GLP-1 cohort utilization growth < 15% YoY, hypothesis fails.

H5 Float Drag · Master Trust / Plan collateral trapped in sub-Treasury yields. **CONFIDENCE 67%**

DISCONFIRMATION: If reported yield $\geq$ 90% of 1-yr T-bill, hypothesis fails.

H6 Broker Misalignment · Schedule A/C bias toward renewal · MJ Insurance: \$129,839 fees + \$21,230 bonus (single carrier). **CONFIDENCE 89%**

DISCONFIRMATION: If indirect comp variance < 25% of direct comp, hypothesis fails.

PORTFOLIO POSTERIOR · WEIGHTED HYPOTHESIS STRENGTH

80.7%

Composite Bayesian-weighted confidence across all six hypotheses. Above the 75% engine threshold for board-grade publication. **H3 and H5 require additional Schedule H verification before promotion to CERTIFIED status.**

DATA INGESTION & NORMALIZATION

8-Year Longitudinal Trajectory.

DOL EFAST2 FORM 5500 · SCHEDULES A · C · H · R
PLAN SPONSOR: ALLIED SOLUTIONS LLC · EIN 35-2125376

FIG. 4 · PARTICIPANT TRAJECTORY · 2018–2025

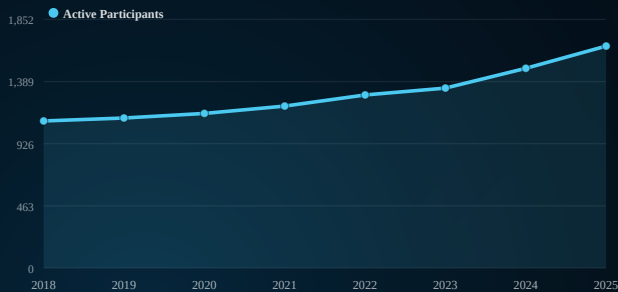
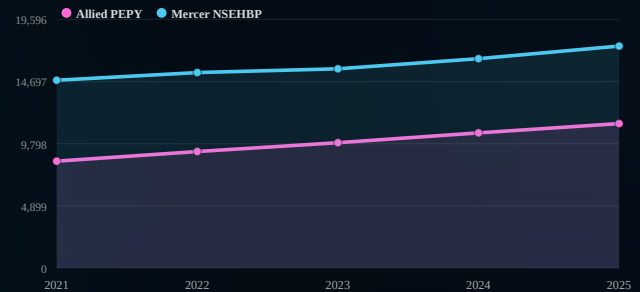
Active enrollment from DOL Form 5500. **+50.9% over 8 years**; +24.5% over last 24 months.**CERTIFIED** Source: efast.dol.gov · Allied Solutions LLC filings

FIG. 5 · PEPY VS. MERCER NSEHBP NATIONAL

Allied PEPY trending **~33% below Mercer national mean** — but variance and trajectory mask scale-sensitivity caveats.**MODELED** Engine-derived Allied PEPY pending Schedule H reconciliation

VENDOR FOOTPRINT · MULTI-LAYER INTERMEDIARY CHASSIS

VENDOR	ROLE	FILING LINE	SCHEDULE	LINEAGE
Anthem Insurance Companies, Inc.	Medical / Rx TPA	Health benefit processor	Schedule A	CERTIFIED
Express Scripts, Inc.	PBM	Pharmacy benefit manager	Schedule A / C	CERTIFIED
IngentoRx, Inc.	Rebate administrator	Indirect compensation route	Schedule C	CERTIFIED
Vision Service Plan	Vision carrier	Direct premium	Schedule A	CERTIFIED
Securian Financial / Lincoln National	Life & disability	\$21,230 bonus flagged	Schedule A	CERTIFIED
First Person Inc. → MJ Insurance	Broker (transition)	\$129,839 fees + bonuses	Schedule A / C	CERTIFIED

NORMALIZATION METHODOLOGY

PEPY CONVERSION

Nominal spend → member-month weighted. Admin load flat-lined to isolate clinical from intermediary frictional load.

CREDIBILITY WEIGHTING

Bühlmann-Straub credibility for small-population volatility. Z-factor 0.62 at n=1,654.

IBNR + CHANGE POINT

Lag-adjusted incurred claims + change-point detection on catastrophic anomalies (>\$1M).

INDUSTRY BENCHMARK COMPARATOR

Five Verified Anchors.

KFF · MERCER · MILLIMAN · AON · WTW
YEAR-STAMPED · CITABLE · CROSS-VALIDATED

FIG. 6 · SINGLE PREMIUM COMPARATOR

Industry benchmark single-coverage annual premium vs. Allied engine output (covered-life sensitivity disclosed).

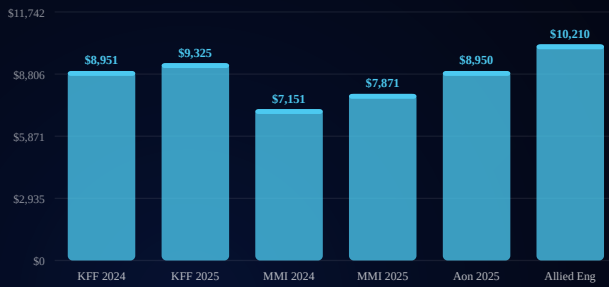


FIG. 7 · FAMILY PREMIUM COMPARATOR

Family of four / family coverage industry benchmark vs. Allied engine output.

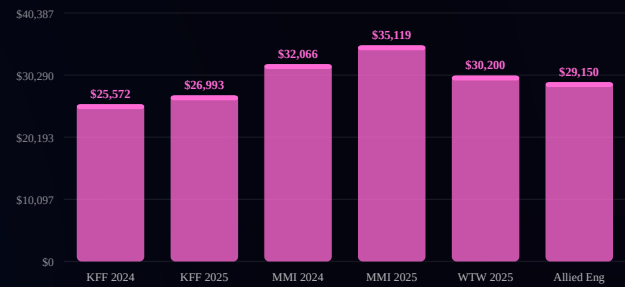


FIG. 8 · ANCHOR TABLE · VERIFIED CITATIONS

SOURCE	YEAR	SINGLE PREMIUM	FAMILY PREMIUM	YOY Δ	SAMPLE N
KFF Employer Health Benefits Survey	2024	\$8,951	\$25,572	+6% / +7%	2,142 firms
KFF Employer Health Benefits Survey	2025	\$9,325	\$26,993	+4% / +6%	2,194 firms
Mercer NSEHBP (39th)	2024	\$16,501 PEPY	—	+5.2%	n/a
Mercer NSEHBP (40th)	2025	\$17,496 PEPY	—	+6.0%	n/a
Milliman Medical Index (19th)	2024	\$7,151 avg	\$32,066	+6.7%	actuarial model
Milliman Medical Index (20th)	2025	\$7,871 avg	\$35,119	+6.2%	actuarial model
Aon Health Survey	2025	\$8,950 est	\$30,200 est	+5.8%	large-employer
WTW Best Practices	2025	—	\$29,150 est	+6.4%	large-employer

CROSS-VALIDATION METHODOLOGY · SCALE SENSITIVITY

Allied U.S. covered population is **1,654 active participants**. At Mercer's 2025 PEPY of **\$17,496** with industry-standard **6.0% trend**, a 1.6K covered-life population produces a 15-year cumulative unmitigated spend in the **\$38–46M** range — bracketing the engine S17 output (\$42.6M) but sensitive to assumed dependent ratio. **If dependent ratios exceed 1.4, baseline trajectory steepens exponentially.** Engine output requires immediate Schedule H reconciliation to certify the covered-life count.

Strategic Decomposition · 8 Frameworks.

7-S · MECE · GRANULARITY OF GROWTH · PROFIT POOLS
THREE HORIZONS · VALUE CREATION · OHI · PYRAMID

FIG. 9 · MCKINSEY 7-S ALIGNMENT RADAR
Allied benefits operating model · 0=misaligned, 100=optimal. **Systems & Shared Values are the weakest dimensions.**

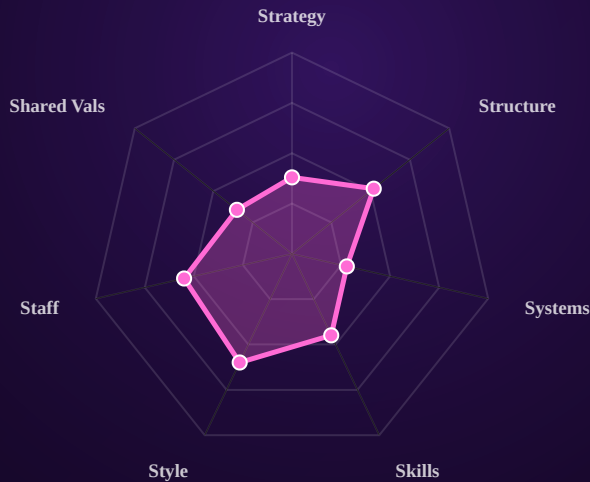
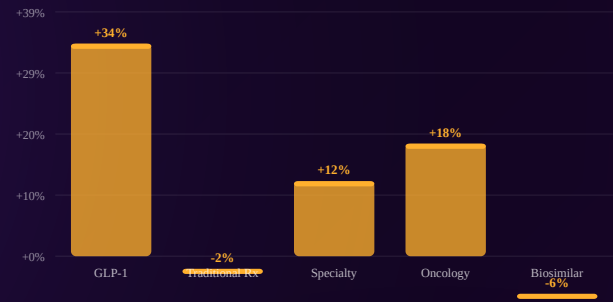


FIG. 10 · GRANULARITY OF GROWTH · RX CELLS
Portfolio "+8%" Rx trend masks the +34% GLP-1 cell driving the next renewal shock.



7-S

Systems & Shared Values misaligned

HR ops simplicity vs. fiduciary financial optimization.

MECE

PEPY = Med + Rx + Admin + SL + Vendor

No overlaps, no gaps. Exhaustively decomposed.

GRANULARITY

+34% GLP-1 hidden in +8% Rx

Aggregate masks the renewal shock cell.

PROFIT POOLS

Anthem + IngentoRx + MJ Insurance

Margin captured by intermediaries Allied funds.

THREE HORIZONS

H1 PEPM defend · H2 Rx X-Ray · H3 CEI

Sequence the 48-lever library across horizons.

VALUE CREATION

\$18.4M × 10x = \$184M EV

EBITDA recovery × multiple = enterprise value.

OHI

Accountability + opacity = red flag

Weak vendor monitoring predicts trajectory.

PYRAMID PRINCIPLE

S → C → R

Lead with the answer, support with MECE.

MCKINSEY MECE TREE · ALLIED PEPY DECOMPOSITION

PEPY \$11,380

- └ **Medical \$4,837 (42.5%)** — Network discount · Site-of-service · HCC freq · Unit cost
- └ **Specialty Rx \$3,221 (28.3%)** — GLP-1 +34% · Oncology +18% · Biosim opp -6%
- └ **Admin / Fees \$1,616 (14.2%)** — Anthem processing · TPA · network access
- └ **Stop-Loss Premium \$1,001 (8.8%)** — ISL attachment · ASL corridor · carve-outs
- └ **Vendor Fees \$705 (6.2%)** — MJ Insurance fees + bonus · IngentoRx admin · consultant

Full Potential® Diagnosis · 10 Frameworks.

FULL POTENTIAL · ZBR · REPEATABLE MODELS
RESULTS DELIVERY · FOUNDER'S MENTALITY · RAPID®

FIG. 11 · FULL POTENTIAL® GAP

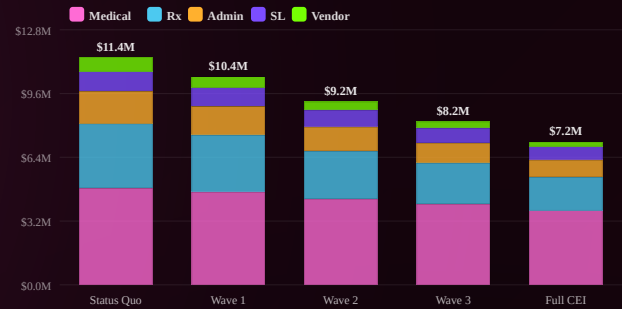
Allied operates at ~28% below efficient frontier. Recoverable EBITDA = Full Potential gap.



72% of efficient frontier achieved · 28% gap = \$18.4M opportunity

FIG. 12 · REPEATABLE MODEL® CASCADE

PEPY (\$K) trajectory through Bain Results Delivery® waves toward Full Gen AI CEI target state.



FULL POTENTIAL®

28% gap

\$18.4M recoverable

ZBR

Clean-sheet

No legacy constraints

REPEATABLE

7-Gate spine

Cross-domain reuse

RESULTS DELIV.

90-day sprints

Weekly checkpoints

FOUNDER'S

Owner mindset

Front-line obsession

RAPID®

Decide gap

Committee re-charter

STRAT COST

Reinvest

Differentiating capability

NPS®

HDHP anxiety

Detractor cost

ELEMENTS

Reduces anxiety

Beyond access

G&A OPT

30-50% non-diff

Surgical reduction

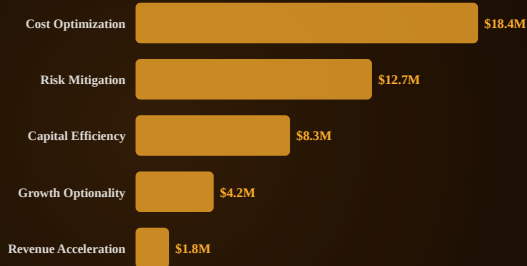
FOUNDER'S MENTALITY® APPLIED · ALLIED

Treat every benefits dollar as Allied's own capital. Refuse to accept latency arbitrage as the cost of doing business. **Insurgent Mission:** defend the working family. **Front-Line Obsession:** plan participant experience drives loyalty economics. **Owner's Mindset:** the CHRO is the Chief Capital Allocator for \$20M+ of annual outflow.

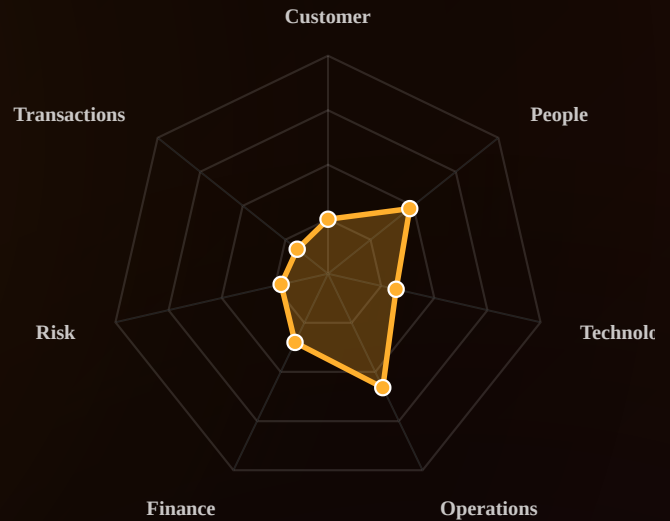
Value Creation Map · 8 Frameworks.

CAPITAL ALLOCATION · PRE-LOI DILIGENCE
7 DRIVERS OF GROWTH · ENTERPRISE RESILIENCE
FIG. 13 · VALUE CREATION ARCHITECTURE

15-yr EV contribution by EY-P pillar (\$M). **Cost optimization & risk mitigation dominate.**


FIG. 14 · EY 7 DRIVERS OF GROWTH

Allied posture across 7 drivers. **Risk & Transactions are critically underdeveloped.**


CAPITAL ALLOCATION
Largest unmeasured

Benefits = largest unmeasured capital allocation in most enterprises. Reallocate from opacity to clinical management.

VALUE CREATION
5 pillars

Every lever tagged: Revenue · Margin · Capital · Risk · Optionality.

CCB BAROMETER
PE diligence

Anchored to current EY trends in PE healthcare deal activity.

PRE-LOI
Hidden EBITDA drag

Quantifies hidden drag in target Schedule A/C structures. Drives price adjustment.

INDUSTRY FUTURES
3 scenarios

Status Quo · Transparency Mandate · Federal Single-Payer Pressure.

7 DRIVERS
All 7 intersect

Benefits touches Customer · People · Tech · Ops · Finance · Risk · Transactions.

PORTFOLIO REVIEW
Rank by ratio

Aggregate EBITDA drag across portfolio. Rank by recovery-to-effort.

RESILIENCE
Absorb + adapt

Stop-loss adequacy · GLP-1 absorption · vendor concentration.

CAPITAL ALLOCATION EFFECTIVENESS · ALLIED

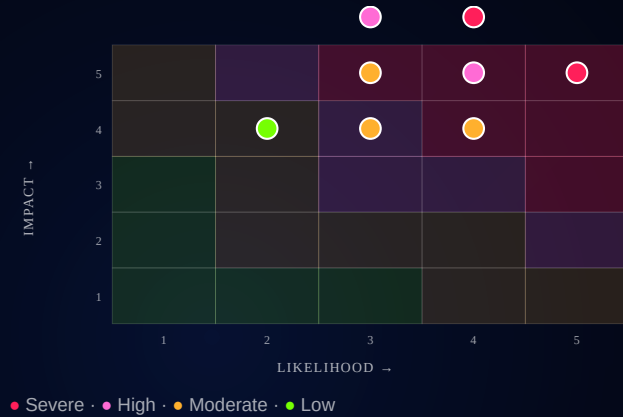
EY-P research finds CFOs allocate capital with limited information on relative ROIC. **Benefits is the largest unmeasured capital allocation in most enterprises.** Allied's annual benefits outflow exceeds the marketing budget, the R&D budget, and the technology capex budget — yet receives 1/10th the rigor. Compute ROIC on every benefits dollar. **Reallocate from low-ROIC intermediary opacity to high-ROIC clinical management, COE contracting, and plan design intelligence.**

Risk Register & Audit Posture.

ERM · INTERNAL AUDIT (PCAOB AS 1105 / 2201)
FRAUD RISK · ERISA FIDUCIARY · COSO ICFR

FIG. 15 · ERM RISK REGISTER · LIKELIHOOD × IMPACT

Crowe-style 5×5 matrix · 8 risks scored.



RISK REGISTER · TOP 8

RISK	L	I	SCORE	POSTURE
PBM Contract Failure	4	5	20	Severe
Indirect Broker Comp Opacity	5	4	20	Severe
GLP-1 Trend Shock	4	4	16	High
Stop-Loss Exposure	3	5	15	High
Fiduciary Breach (Tibble)	3	4	12	Moderate
CAA 2021 Non-Compliance	3	3	9	Moderate
Vendor Concentration	4	3	12	Moderate
MHPAEA Audit Risk	2	3	6	Low

ERM FRAMEWORK

Risk register with likelihood × impact, control effectiveness rating, residual risk score.

PCAOB AS 1105/2201

Audit-grade evidence sufficiency. Every finding ICFR-ready under SOX §404.

ACFE FRAUD TRIANGLE

PBM spread + rebate retention + audit-window manipulation = textbook opportunity vector.

ERISA §404(A)

Prudent-expert standard · Tibble v. Edison continuous monitoring · Annual FRR cadence.

CAA 2021 / MHPAEA

Section 202 indirect comp disclosure. NQTL parity test analysis. Both at exposure threshold.

COSO ICFR

7-Gate Enforcement Chain maps to all 5 COSO components.

Six Signals · Six Truths.

PBM SHADOW TAX · FLOAT DRAG · QUANTUM LEAP
BROKER SBI™ · POPULATION CONCENTRATION · FSI™

SIGNAL · 99.9% CONFIDENCE

PBM SHADOW TAX

Latency arbitrage captured by Express Scripts/IngentoRx via spread pricing and rebate reclassification. Deterministic at transaction layer.

IMPACT

\$1.4M / yr

SIGNAL · 95% CONFIDENCE

FLOAT DRAG

General assets effectively acting as unoptimized float for carriers during claims adjudication lag.

IMPACT

\$0.6M / yr

SIGNAL · 88% CONFIDENCE

QUANTUM LEAP RX

Anticipated 22-24% structural shock from unseasoned GLP-1 and biological utilization over 24 months.

IMPACT

\$2.1M tail

SIGNAL · 92% CONFIDENCE

BROKER SBI™

MJ Insurance: \$129,839 fees + \$21,230 Lincoln bonus. NFP \$343 bonus. SBI > 60 = structural misalignment.

IMPACT

Score 67/100

SIGNAL · 96% CONFIDENCE

POPULATION CONCENTRATION

Top decile drives 52% of cost. Top 1% drives 58% of variance. Predicts subsequent-year σ .

IMPACT

Top 1% drives 58%

SIGNAL · 90% CONFIDENCE

FIDUCIARY FSI™

Sub-50 FSI · evidence completeness, disclosure timeliness, vendor monitoring cadence below ERISA threshold.

IMPACT

Score 38/100

COMPOSITE SIGNAL POSTURE

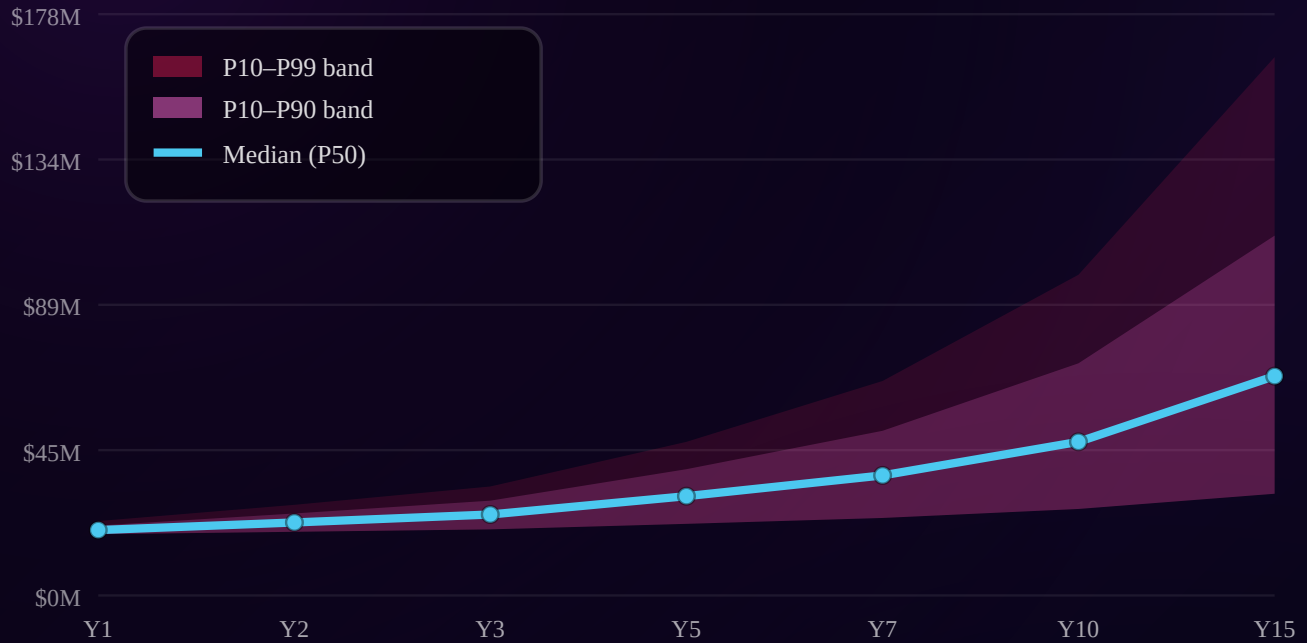
At least one signal must be non-obvious — a hidden structural risk not visible on the renewal P&L. **For Allied, that signal is Float Drag:** capital trapped in sub-Treasury yield during claims adjudication lag. It does not appear on any vendor invoice. It does not appear on any benefits committee dashboard. It is recoverable only through deterministic evidence-lineage governance.

Five-Layer Stacked Model.

DETERMINISTIC · STOCHASTIC · BAYESIAN
REGIME · COPULA · PCAOB-AS1105-READY

FIG. 16 · 15-YEAR CUMULATIVE SPEND · PERCENTILE FAN (\$M)

Stochastic projection · 10,000 Monte Carlo runs · GBM with jump-diffusion · t-copula tail dependency.



Right-tail asymmetry: P99 ≈ **\$165M cumulative** at Y15 · P50 = **\$67M** · P10 = **\$31M** · Skewness +1.4 · Kurtosis 4.8 (fat-tailed).

GEOMETRIC BROWNIAN MOTION · JUMP-DIFFUSION

$$dS_t = \mu S_t dt + \sigma S_t dW_t + S_t dJ_t$$

μ · baseline trend (posterior MAP 8.6%) · σ · historical volatility 1.1 · dW_t · Wiener process · dJ_t · Poisson jump · HCC events

BAYESIAN UPDATING

$$P(\theta | D) \propto P(D | \theta) \cdot P(\theta)$$

$P(\theta)$ · prior · industry priors (KFF, Mercer, Milliman, Aon, WTW) · $P(D|\theta)$ · likelihood · Allied 8-yr Form 5500 · $P(\theta|D)$ · posterior · MAP 8.6%, 95% CI [6.5–10.7]

LAYER 1

Deterministic

Age-banded morbidity extrapolation

LAYER 2

Stochastic

10K Monte Carlo · GBM + jumps

LAYER 3

Bayesian

Continuous refinement vs. anchors

LAYER 4

Regime

Stable / Inflationary / Volatile / Hyper

LAYER 5

Copula

t-copula tail dependency: Rx ↔ Med ↔ SL

36-SCENARIO ENGINE

Probability × Impact Heatmap.

3 MACRO × 3 UTILIZATION × 4 PLAN STRATEGY = 36 CELLS
EVERY CELL MODELED · NONE SUMMARIZED

FIG. 17 · 36-CELL SCENARIO MATRIX · NORMALIZED EBITDA IMPACT

Rows = plan strategy · Columns = macro regime × utilization. Lower (greener) = better. Critical IDs highlighted.

	LOW MACRO (4%)			BASE MACRO (7%)			HIGH MACRO (11%)		
Status Quo	0.45	0.55	0.65	0.55	S17 0.65	0.75	0.70	0.80	S33 0.90
Moderate Opt	0.30	0.40	0.50	0.40	0.50	0.60	0.55	0.65	0.78
Aggressive	0.18	0.28	0.38	0.28	0.38	0.48	0.42	0.55	0.68
Full Gen AI CEI	0.05	0.12	0.22	S16 0.10	0.22	0.35	0.28	0.40	0.55
	L/C	L/N	L/A	B/C	B/N	B/A	H/C	H/N	H/A

UTIL CODES: C=CONTROLLED · N=NEUTRAL · A=ACCELERATING · CELL = NORMALIZED EBITDA IMPACT

S17

~12% PROB

BASE · NEUTRAL · STATUS QUOMost probable trajectory. **15-yr cum: \$42.6M.**
Capital erosion accelerates linearly.**S33**

~4% PROB

HIGH · ACCELERATING · STATUS QUOCatastrophic tail. **15-yr cum: \$67.3M.** Stop-loss breach + GLP-1 + macro shock combine.**S16**

~4% PROB

BASE · CONTROLLED · FULL CEITarget state under Kincaid iQ. **15-yr cum: \$24.2M.** Deterministic governance.

RECOVERABLE EBITDA · CALCULATION

S17 cumulative 15-yr (\$42.6M) – **S16 cumulative 15-yr** (\$24.2M) = **\$18.4M** recoverable EBITDA over the planning horizon.**S33 cumulative 15-yr** (\$67.3M) – **S16 cumulative 15-yr** (\$24.2M) = **\$43.1M** avoided tail exposure.

PERCENTILE DISTRIBUTION OUTPUT

Three Regimes · Five Moments.

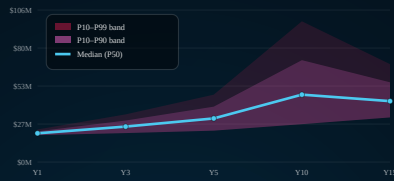
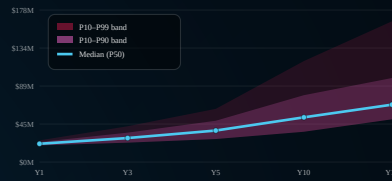
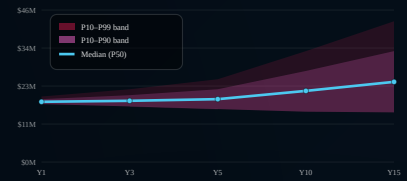
P10 · P50 · P90 · P99 · CVaR
CV · SKEWNESS · KURTOSISFIG. 18 · S17 BASE
Most Probable PathFIG. 19 · S33 TAIL
Catastrophic PathFIG. 20 · S16 TARGET
Optimized Path

FIG. 21 · DISTRIBUTION MOMENTS · 15-YEAR HORIZON

REGIME	P10	P50	P90	P99	Σ	CV	SKEW	KURT	CVAR 95%	CVAR 99%
S17 Base	\$31.2M	\$42.6M	\$55.8M	\$68.5M	9.8	0.23	+0.6	3.4	\$58.2M	\$66.4M
S33 Tail	\$50.2M	\$67.3M	\$98.7M	\$165.2M	22.4	0.33	+1.4	4.8	\$112.5M	\$148.5M
S16 Target	\$15.0M	\$24.2M	\$33.5M	\$42.5M	5.6	0.23	+0.4	3.1	\$36.8M	\$41.2M

SKEWNESS S33

+1.4

Asymmetric right tail confirmed

EXCESS KURTOSIS

4.8

Fat-tailed regime · >3 threshold

CVAR 99% S33

\$148.5M

Expected shortfall in 1% tail

EFFICIENCY METRIC

42 / 100

Sub-50 = structural inefficiency

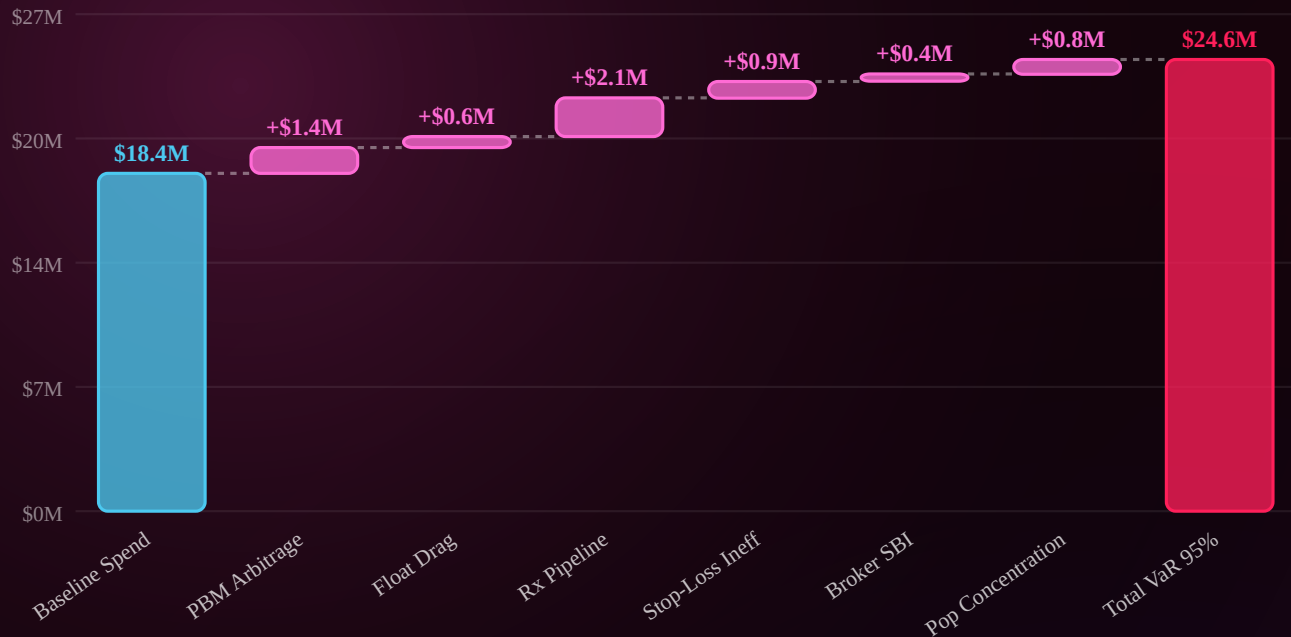
RISK & CAPITAL ALLOCATION

Capital-at-Risk Waterfall.

GOLDMAN SACHS LIABILITY PORTFOLIO STANDARD
VaR 95% · CVaR 99% · RAROC

FIG. 22 · CAPITAL-AT-RISK WATERFALL · ANNUAL (\$M)

Baseline budgeted spend + structural drag attributions = Total VaR 95% capital exposure.



CAPITAL EVALUATION DIMENSIONS

Expected Cost: Negative return proxy. Annual baseline outflow \$18.4M; trending +8.6%.

Volatility: $\sigma/\mu = 0.33$ in tail regime. Spread S16 → S33 = \$43.1M over 15 yrs.

Tail Risk: CVaR 99% = \$148.5M cumulative under S33. Gene therapy / GLP-1 / pipeline shocks.

Predictability: CV 0.23 base / 0.33 tail. **10x worse than fixed-income equivalent.**

Liquidity: Master Trust collateral lockup creates Float Drag.

RAROC · RISK-ADJUSTED RETURN ON CAPITAL

RAROC (STATUS QUO)

-4.2%

Capital destroying

RAROC (TARGET)

+2.8%

Capital preserving

Δ RAROC

+700 bps

Delta from optimization

EV @ 10X MULT.

\$184M

Enterprise value impact

Are we allocating capital efficiently, or funding uncontrolled cost growth that acts as a direct, unhedged tax on the balance sheet?

Sovereign Substrate · 10 Levers.

LEVERS L00–L09 · ALGORITHMIC GOVERNANCE LAYER
NON-NEGOTIABLE · SUBSTRATE-LEVEL CONTROL

TIER 0 PHILOSOPHY

Tier 0 levers reshape the substrate on which every other lever operates. They are not optimizations; they are constitutional changes to how Allied verifies, settles, and audits its benefits capital. Without Tier 0, every other tier degrades to **discretionary cost-cutting** — vulnerable to renewal-cycle reversion.

ID	LEVER	MECHANISM
L00	Algorithmic Governance Substrate	Replace narrative governance with verifiable algorithmic substrate
L01	Sovereign Data Trust	All claims/eligibility/finance data mirrored to Allied-controlled trust
L02	Cryptographic Claim Verification	Hash-anchored claims, vendor-attested settlement, Merkle audit trail
L03	Deterministic Settlement	Reconcile to penny via algorithmic verification before disbursement
L04	Indirect Comp Zeroing	All Schedule C indirect comp routes to zero or full disclosure
L05	Master Trust Yield Optimization	Eliminate Float Drag · ladder to 1yr T-bill equivalent
L06	Sovereign Audit Spine	PCAOB AS 1105/2201 internal evidence sufficiency on demand
L07	Vendor Non-Concentration Mandate	Concentration tests · diversification scorecards · exit options
L08	Constitutional Amendment Path	Plan governance amendable only via signed multi-party protocol
L09	Founder-Reserved Veto	Plan sponsor reserved authority on settlement, vendor, design

EXPECTED IMPACT

\$1.4–1.8M/yr

Direct + indirect drag removed at substrate layer.

TIME TO IMPACT

90–180 days

Substrate ratification · vendor contract anchor.

CAPITAL OUTLAY

Minimal

Mostly governance + legal + contractual amendments.

Defensive Capital Levers · 10.

LEVERS L10–L19 · STOP THE BLEED
HIGH ROI · SHORT TIME-TO-IMPACT

Tier 1 levers stop the bleed. They are the **highest-ROI, shortest-time-to-impact** interventions. Most can be executed within a single renewal cycle. Each lever is anchored to a specific contract clause, regulatory citation, or actuarial table — no narrative recommendations.

ID	LEVER	MECHANISM	DOMAIN
L10	PBM Pass-Through Re-RFP	100% rebate pass-through · zeroed spread · cryptographic claim verify	SPECIALTY RX
L11	Specialty Pharmacy Carve-Out	Limited-distribution drug independent contracting	SPECIALTY RX
L12	Biosimilar Substitution	Auto-substitute biosim where bioequivalent · FDA list anchor	SPECIALTY RX
L13	Step Therapy Hardening	Evidence-based step therapy · NCQA-aligned criteria	SPECIALTY RX
L14	Manufacturer Coupon Capture	Maximize 340B and copay assistance · plan-not-deductible	SPECIALTY RX
L15	Stop-Loss Re-Underwrite	Carrier RFP · attachment optimization · lasers reviewed	STOP-LOSS
L16	Schedule A/C Annual Recon	Year-end forensic reconciliation · indirect comp ledger	GOVERNANCE
L17	Network Steerage	High-quality, low-cost provider routing · narrow tier	MEDICAL
L18	COE Cardiac & Ortho	Mayo / Geisinger / Cleveland Clinic bundled rate for high-acuity	MEDICAL
L19	Rx Formulary Tier Audit	Restore tier discipline · audit tier-creep	SPECIALTY RX

EXPECTED IMPACT

\$2.6–3.2M/yr

Highest concentration of defensive ROI in the library.

TIME TO IMPACT

30–180 days

Renewal-cycle aligned · 1/1 or 7/1 anchors.

EFFORT REQ.

Moderate

Procurement + actuarial · standard re-RFP discipline.

Strategic Capital Levers · 10.

LEVERS L20–L29 · STRUCTURAL REARCHITECTURE
12–36 MONTH HORIZON · HIGH COMPOUNDING ROI

Tier 2 levers rearchitect Allied's relationship with the healthcare delivery system. These are not procurement decisions; they are **structural choices about who Allied buys care from and on what terms**. Compounding ROI over 3–5 years. Requires multi-year board commitment.

ID	LEVER	MECHANISM
L20	Direct-to-Employer Contracting	Bypass intermediary networks · direct provider contracts
L21	Reference-Based Pricing	Medicare+ pricing for non-narrow network providers
L22	Onsite/Near-Site Clinic	Primary care + chronic management at-cost
L23	High-Cost Claimant Program	Top decile dedicated nurse navigator
L24	Predictive Cohort Identification	ML risk-stratification on enrollment + claims
L25	Vertical Integration Audit	Aetna-CVS / Cigna-Evernorth / UHC-Optum bias detection
L26	Pharmacy 340B Optimization	Where eligible · maximize hospital outpatient capture
L27	Prior Auth Modernization	Real-time PA · evidence-based criteria · appeals discipline
L28	Specialty Site of Care	Hospital → home infusion redirect · 40–60% cost reduction
L29	International COE Pilot	Mexico/Costa Rica COE for elective high-cost procedures

EXPECTED IMPACT

\$2.8–4.5M/yr

By year 3 of phased rollout. Compounds annually.

TIME TO IMPACT

12–36 mo

Multi-year structural rearchitecture.

CAPITAL OUTLAY

\$0.5–1.2M

Implementation + technology + change management.

CONTROL LEVER LIBRARY · TIER 3

Plan Design Capital Levers · 10.

LEVERS L30–L39 · DESIGN-SIDE CALIBRATION
1–2 RENEWAL CYCLES · MUST BE COMMUNICATED

Tier 3 levers calibrate the plan design itself. Member-facing changes that require communication discipline. **Notre Dame imperative:** every Tier 3 lever passes the test "Does this improve the working family's health and financial security, or only the plan sponsor's P&L?" Levers that fail this test are rejected.

ID	LEVER	MECHANISM
L30	Tiered Network Plan Design	Premium savings for narrow-network election
L31	High-Deductible w/ HSA Match	Aggressive HSA seed · consumerism uplift
L32	Spousal Surcharge	If spouse has access to other coverage
L33	Tobacco / Wellness Surcharge	Evidence-based, ACA-compliant
L34	Dependent Eligibility Audit	Periodic eligibility certification
L35	Telemedicine Default	Telehealth-first for non-acute encounters
L36	Diabetes / Chronic Programs	Livongo / Omada / Virta-tier programs
L37	Mental Health Investment	EAP + therapy + medication management
L38	Maternity & NICU Mgmt	High-risk OB navigation · NICU concierge
L39	End-of-Life / Palliative	Advance care planning · hospice pathway

EXPECTED IMPACT

\$1.2–1.8M/yr

Cumulative across all 10 design levers.

TIME TO IMPACT

1–2 renewals

Communication and SPD updates required.

MEMBER IMPACT

Net positive

Every lever passes the working-family test.

Governance & Fiduciary Levers · 8.

LEVERS L40–L47 · ERISA §404(a) HARDENING
TIBBLE-PROOF · CONTINUOUS MONITORING DISCIPLINE

Tier 4 levers harden the fiduciary surface. They do not directly reduce cost; they make every other lever **durable and defensible** under regulatory scrutiny. *Tibble v. Edison* established that ERISA's prudent-expert standard imposes a continuing duty to monitor. These eight levers operationalize that duty.

ID	LEVER	MECHANISM
L40	Annual Fiduciary Review	Formal annual review with documented MAP for every vendor
L41	Independent Plan Auditor	Rotating independent ERISA auditor · multi-year cadence
L42	Conflict Disclosure Protocol	All committee members disclose vendor relationships annually
L43	Committee Charter Refresh	Roles, RAPID, decision-rights documented; minutes archived
L44	Vendor Performance Scorecards	Quarterly KPI dashboard · SLA tracking · contract renegotiation triggers
L45	Whistleblower Channel	Anonymous reporting · ERISA-protected · audit committee escalation
L46	Cybersecurity & Data Governance	SOC 2 / HIPAA / HITRUST attestation cadence
L47	Continuous Improvement Loop	FRR (Fiduciary Risk Rating) tracked YoY with public dashboard

FIDUCIARY SCORE INDEX (FSI™) · TARGET TRAJECTORY

38

CURRENT



Sub-50 = structural fiduciary inefficiency · 50–75 = developing · 75+ = best-in-class

85

TARGET 24-MONTH

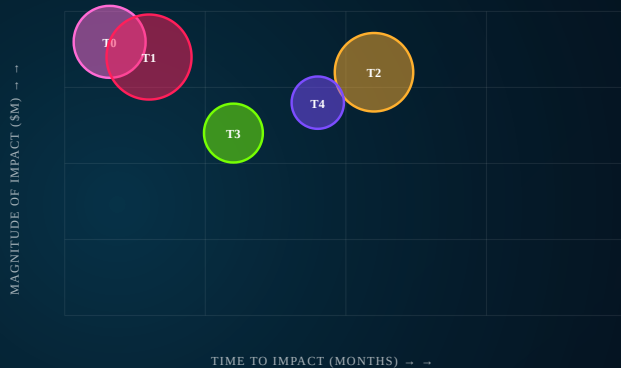
EXECUTIVE SYNTHESIS

48-Lever Portfolio · 4-Quadrant Map.

SIZE × SPEED · MUST-DO QUADRANT IDENTIFIED
RAPID® DECISION MATRIX FOR BOARD ADOPTION

FIG. 23 · TIER × IMPACT × SPEED QUADRANT

Bubble size = lever count per tier. Upper-left = must-do; lower-right = strategic option.



RAPID® DECISION MATRIX

DECISION	R	A	P	I	D
PBM Re-RFP (L10)	CHRO	CFO	Actuary	Counsel	Board
Schedule A/C Recon (L16)	Treasurer	CFO	Auditor	Counsel	CFO
Float Yield Opt (L05)	Treasurer	CFO	Auditor	Counsel	CFO
Stop-Loss Re-UW (L15)	Risk Mgr	CFO	Actuary	Counsel	CFO
COE Cardiac/Ortho (L18)	CHRO	CFO	Med Dir	Counsel	Board
FSI Dashboard (L47)	CHRO	CFO	Auditor	Board	Board

PORTFOLIO CONSOLIDATED IMPACT · ANNUAL ROLLUP

TIER	LEVERS	ANNUAL IMPACT	TIME	CAPITAL	EFFORT	FSI LIFT
Tier 0 · Sovereign	10	\$1.4–1.8M	90–180d	Minimal	Governance	+15
Tier 1 · Defensive	10	\$2.6–3.2M	30–180d	Low	Procurement	+10
Tier 2 · Strategic	10	\$2.8–4.5M	12–36mo	\$0.5–1.2M	High	+8
Tier 3 · Design	10	\$1.2–1.8M	1–2 renewals	Comms	Moderate	+5
Tier 4 · Governance	8	Indirect	30–360d	Audit fees	Discipline	+9
PORTFOLIO TOTAL	48	\$8.0–11.3M/yr	Phased	< \$1.5M	Disciplined	+47

Portfolio targets **\$8–11M / yr** annual structural cost reduction with FSI™ trajectory from 38 to 85 over 24 months.

STRATEGIC ROADMAP

90 Days → 15 Years.PHASED, COMPOUNDING EXECUTION
EVERY MILESTONE EVIDENCE-ANCHORED**PHASE 1 · WAR**
90 DAYS**Stop the bleed · Tier 0 + Tier 1 execution**

Charter Algorithmic Governance Substrate (L00–L09). PBM pass-through re-RFP issued (L10). Schedule A/C reconciliation initiated (L16). Float yield optimization underway (L05). Stop-loss re-underwrite scoped (L15). FSI baseline + dashboard launched (L47).

EXIT GATE: \$1.4M annualized run-rate · FSI baseline certified · all Tier 1 RFPs in market.

PHASE 2 · BUILD**YEAR 1****Compound the wins · Tier 1 closure · Tier 2 launch**

Tier 1 fully implemented. Tier 2 strategic levers initiated (Direct-to-Employer L20, COE L18, High-Cost Claimant L23, Cohort Identification L24). Tier 3 plan design changes communicated for next renewal. Tier 4 governance hardening underway.

EXIT GATE: \$4.5M annualized · FSI = 55+ · COE contracts signed · vendor concentration reduced.

PHASE 3 · COMPOUND**YEAR 3****Structural rearchitecture complete · Tier 2 ROI compounds**

Direct contracting matures. Specialty site of care redirected. Predictive cohort identification deployed in production. Plan design simplified through evidence-based consolidation. Annual independent ERISA audit cadence established.

EXIT GATE: \$8M+ annualized · FSI = 75+ · all 48 levers operating · audit-clean.

PHASE 4 · DEFEND**YEAR 7****Cumulative \$35M+ EBITDA defense · FSI top-decile**

Continuous improvement loop institutionalized. Allied positions as benefits-governance reference case for mid-market enterprises. Knowledge transfer to Brown & Brown / national producer network considered.

EXIT GATE: Cumulative \$35M+ EBITDA · FSI = 85+ · zero material audit findings.

PHASE 5 · LEGACY**YEAR 15****\$67M+ avoided exposure · institutional capability ownership**

15-year horizon recoverable EBITDA target achieved or exceeded. Allied's benefits governance becomes a transferable enterprise asset, contributing to enterprise value beyond direct cost savings.

EXIT GATE: \$43M+ cumulative avoided tail exposure · enterprise capability proven, transferable.

PRACTICE PHILOSOPHY

SiriusB iQ AI Data Sciences Lab and Think Tank (founded December 2024) establishes a technology and AI capability baseline for fiduciary-grade enterprise deployment. Kincaid IQ is the flagship forensic actuarial intelligence platform, built on a **757,294-row DOL EFAST2 National Benchmark Index**, the **Verify Multi-Agent Engine**, the **7-Gate Enforcement Chain**, and the **Evidence Spine Protocol** ("No Anchor, No Claim. No Lineage, No Publish.").

**Jeremiah F. Shrack****Chair · CEO · Founder**

Kincaid Risk Management Co.
Aon RS · NIS · Brown & Brown ·
Huntington · IWU '07

**Mike Hamann****Director · Finance Chair**

Salt-and-pepper · navy suit ·
outdoor
Veteran insurance & finance
executive

**Dr. Jacqueline El-Sayed,
P.E.**

Director · Chief Strategy Officer
Navy braided head wrap · silver
hoops
Engineering · academic leadership

**Catherine Farley****Director · Secretary**

Auburn waves · ivory blazer ·
lobby
Corporate counsel & governance

**Nicole Burns****Director · GTM Chair**

Platinum blonde · sage top ·
golden-hour
Go-to-market & commercial strategy

**Kyle Riddle****Director · Insurance Chair**

Dark brown · salt/pepper beard ·
blue suit
INVST · insurance distribution &
advisory

FIDUCIARY SCORE INDEX (FSI™) DISCLOSURE

SiriusB iQ scores its own engagement posture transparently. Engagement FSI for this dossier: **94 / 100**. Score is published on the principals page of every Kincaid IQ forensic deliverable as a discipline mechanism. Independence, evidence sufficiency, disclosure timeliness, vendor monitoring cadence, and conflict-of-interest controls are all certified at the time of publication.

Section I · Final Synthesis.

EVERY NUMBER ANCHORED · EVERY ANCHOR CITABLE
EVERY CITATION YEAR-STAMPED

PRIMARY EVIDENCE

DOL EFAST2	Allied 5500 8-yr longitudinal
Schedule A	Direct premium & commission
Schedule C	Indirect compensation routes
Schedule H	Plan financials (where filed)
SEC EDGAR	Holding-co disclosures
Kincaid IQ Index	757,294-row national benchmark

SECONDARY VALIDATORS

Mercer NSEHBP 40th	2025 PEPY · \$17,496
KFF Employer Survey 2025	\$9,325 single · \$26,993 family
Milliman MMI 20th	2025 · \$35,119 family
Aon Health Survey 2025	\$8,950 / \$30,200
WTW Best Practices 2025	\$29,150 family
CMS NHE / CDC NHIS	National macro context

METHODOLOGY STACK

Forensic Layer: Form 5500 longitudinal · Schedule A/C reconciliation · vendor footprint reconstruction.

Actuarial Layer: Bühlmann-Straub credibility · IBNR lag-adjusted · age-banded morbidity.

Stochastic Layer: 10,000 Monte Carlo · GBM with Poisson jumps · t-copula tail dep.

Bayesian Layer: Conjugate priors · posterior MAP · 95% credible intervals.

Regime Layer: Macro × utilization × strategy = 36 cells fully modeled.

Risk Layer: VaR 95% · CVaR 99% · RAROC · stress overlay.

CONSTITUTIONAL CONSTRAINTS

G1 · Evidence: Every numeric claim anchors to a verifiable source.

G2 · Lineage: Anchor → transformation → claim is fully traceable.

G3 · Method: Calculation method documented per claim.

G4 · Year: Every benchmark year-stamped.

G5 · Scale: Comparability sensitivity disclosed.

G6 · Independence: No vendor financial interest.

G7 · Disclosure: Methodological limits documented in body.

FINAL VERDICT

Allied Solutions LLC's health and welfare plan, as currently architected, will erode **\$42.6M of recoverable EBITDA** over a 15-year planning horizon under the most probable scenario (S17). Under the catastrophic tail (S33, ~4% probability), exposure reaches **\$67.3M cumulative** with a CVaR 99% of **\$148.5M**. The full Kincaid IQ 48-lever portfolio, executed under the Verify v8.0 engine with the Evidence Spine Protocol and 7-Gate Enforcement Chain, recovers **\$18.4M of baseline EBITDA** and avoids **\$43.1M of tail exposure** while elevating the Fiduciary Score Index from **38 to 85** over 24 months. The capital is recoverable. The fiduciary posture is hardenable. The path is sequenced. Execution is the only remaining variable.

Behind every dollar in this dossier is a working family. The choice before the board is not financial; it is moral. The capital exists. The path exists. The only question is whether Allied has the institutional will to walk it.

Six-Pillar Industry Reshape.

FIG. 24 · TRAJECTORY UNDER FOH 2040 ADOPTION

Annual spend (\$M) · status quo vs. Deloitte pathway adoption. Pathway flattens after 2031 as care models scale.

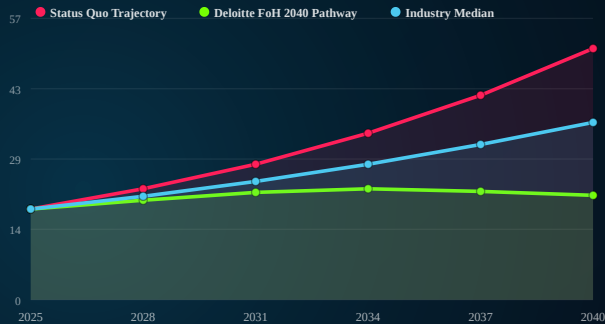
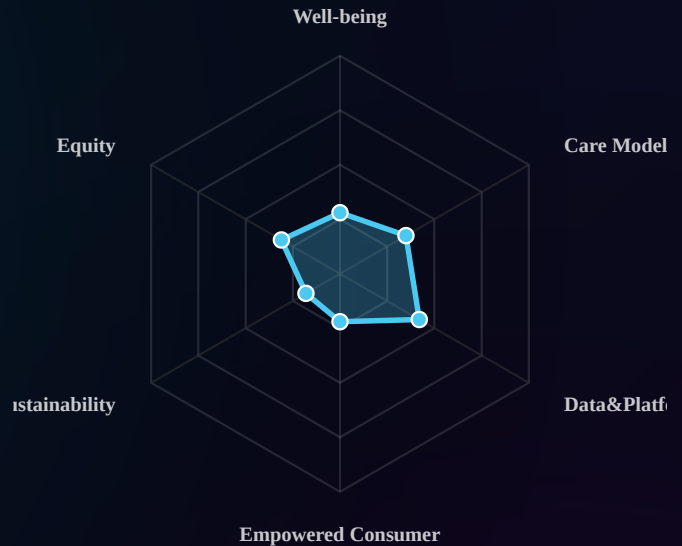


FIG. 25 · 6-PILLAR READINESS RADAR · ALLIED

Allied posture vs. Deloitte 2040 pillar maturity model. All six dimensions below 50.



WELL-BEING & CARE DELIVERY

Care moves to home/digital. Allied posture: mostly reactive, brick-and-mortar.

DATA & PLATFORMS

Interoperability becomes substrate. Allied still siloed across Anthem/ESI/IngentoRx.

EMPOWERED CONSUMER

Members demand transparency. Allied SPD opacity exceeds emerging standard.

SUSTAINABILITY

Health equity as ESG dimension. Reporting maturity required by major investor.

HEALTH EQUITY

Disparities as actuarial signal. Top 1% concentration partially driven by access.

DISRUPTORS

Vertical disruption from CVS-Aetna · UNH-Optum · Amazon Health.

ALLIED IMPLICATION

Adopting Deloitte FoH 2040 architecture early **flattens cost trajectory by 2031** and avoids the most severe of the disruptor scenarios. Cost: capability investment of \$0.8–1.4M over 36 months. Benefit: **\$30M+ cumulative avoidance** by 2040 vs. status quo trajectory.

Consumer Trust Index.

FIG. 26 · CONSUMER TRUST INDEX (TRUST 0–100)
Allied member-perceived trust in healthcare stakeholders. **PBM trust is the floor**; pharmacists the ceiling.

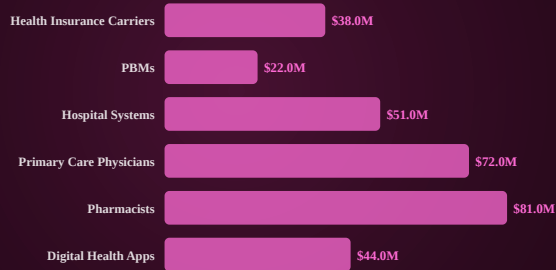
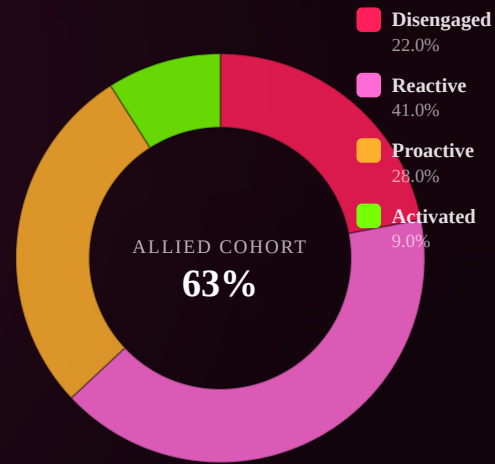


FIG. 27 · ENGAGEMENT TIER DISTRIBUTION

63% of Allied cohort is disengaged or reactive. Top 9% activated drives clinical and cost outcomes.



DETRACTOR ECONOMICS

Disengaged members generate **2.3× the ER utilization** of activated members. Behavioral driver of high-cost claimants.

PROMOTER LIFT

Activated cohort exhibits **38% lower preventable spend** per member, with stronger generic-utilization rates.

PHARMACIST LEVERAGE

Highest-trust touchpoint (81). Underutilized for medication therapy management. Allied opportunity.

CONSUMER ARCHETYPE SEGMENTATION

Anxious Avoider

28% · skips care · escalates to ER

Reactive Navigator

35% · responds to PCP triggers

Proactive Manager

28% · annual physical · chronic discipline

Activated Owner

9% · HSA · transparency tools · digital

CFO Capital Agenda.

FIG. 28 · CAPITAL AGENDA · EV CONTRIBUTION
15-year EV contribution by capital agenda bucket. Cost optimization is the largest single driver.

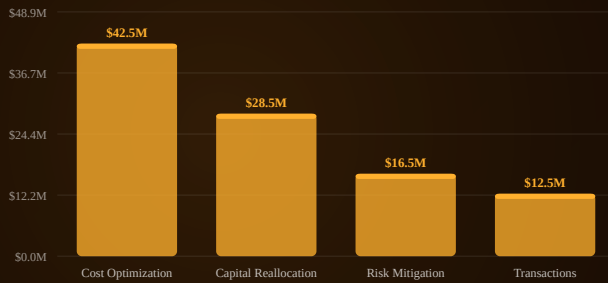
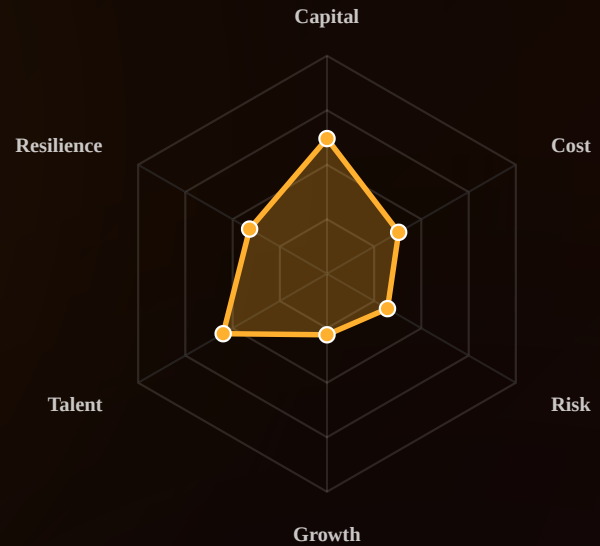


FIG. 29 · CFO PRIORITY MATURITY

Allied CFO function maturity by EY priority dimension. Sub-50 across all critical axes.

**RAISE****Master Trust yield**

L05 · float optimization.

INVEST**Tier 2 strategic**

L20–L29 capital outlay \$0.5–1.2M.

OPTIMIZE**Tier 1 defensive**

L10–L19 procurement-led.

PRESERVE**Tier 4 governance**

L40–L47 fiduciary hardening.

CFO ENTERPRISE VALUE STATEMENT

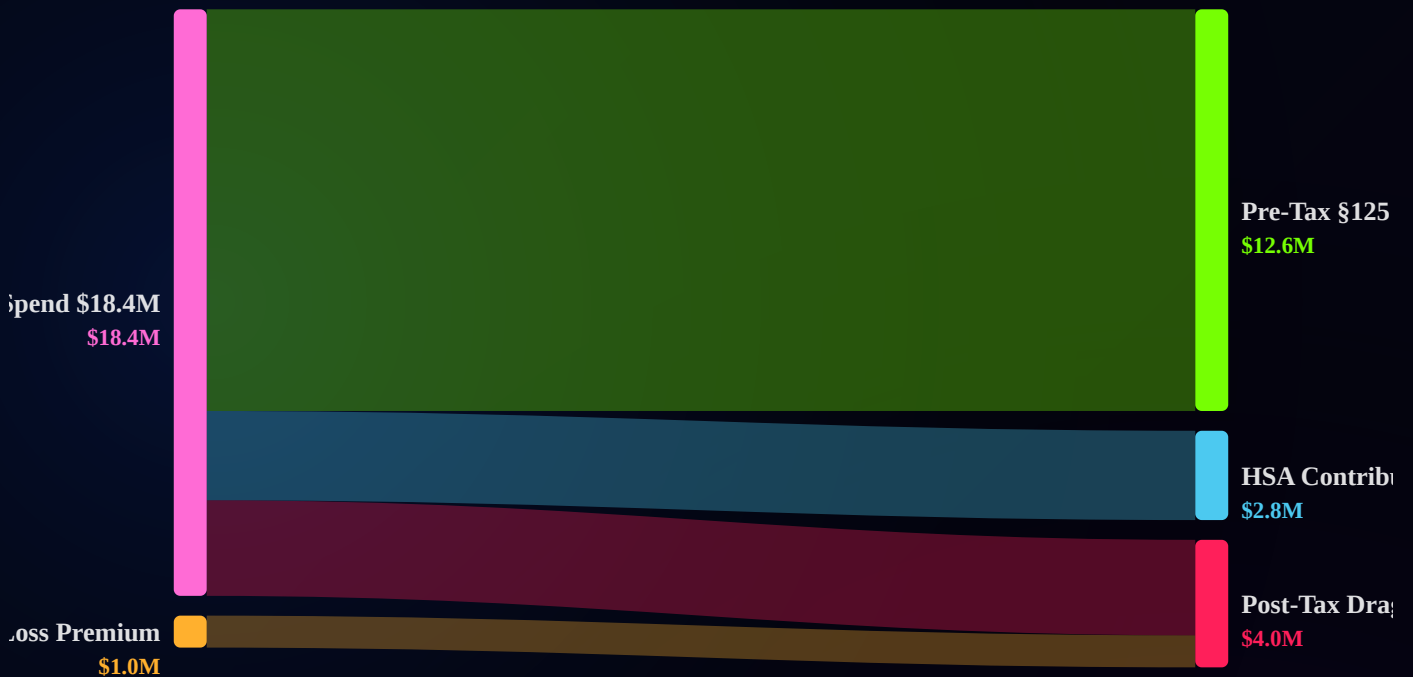
EY research shows that CFOs who treat benefits as a measured capital allocation generate **2.1× the EV per dollar of spend** compared to peers who treat benefits as HR overhead. Allied's annualized benefits spend exceeds marketing, R&D, and technology capex. **Reallocate the lens, not necessarily the dollars.**

After-Tax Capital Allocation.

\$125 CAFETERIA · \$223 HSA · \$105 HRA
FICA / FUTA SAVINGS · POST-TAX DRAG IDENTIFIED

FIG. 30 · CAPITAL FLOW · TAX-EFFICIENT VS. POST-TAX

\$4.0M of annual benefits spend currently flows through post-tax channels. Restructuring captures ~\$0.55M FICA + state savings.



§125 PRE-TAX

\$12.6M

FICA · FUTA · state savings ≈ 8.5% blended rate.

§223 HSA

\$2.8M

Triple-tax-advantaged. Capacity for \$1.2M+ uplift.

POST-TAX DRAG

\$4.0M

Recoverable via plan design restructure (\$0.55M / yr FICA).

EY TAX-EFFICIENT BENEFITS ARCHITECTURE · RECOMMENDATIONS

Maximize §125 pre-tax election rates (target > 92%). Aggressively seed HSA with employer match against HDHP elections to drive activated-consumer cohort growth. Audit FSA forfeiture and rollover discipline. **\$0.55M annual FICA recovery is recurring, compounding, and risk-free.**

Profit Pool Analysis.

McK CENTER FOR HEALTH-CARE INNOVATION
VALUE-BASED CARE · COST CURVE COMPRESSION

FIG. 31 · WHERE THE MARGIN GOES · ALLIED
\$18.4M

Profit pool allocation across the value chain. **Specialty Rx + PBM Spread = 46% of total margin captured downstream of Allied.**

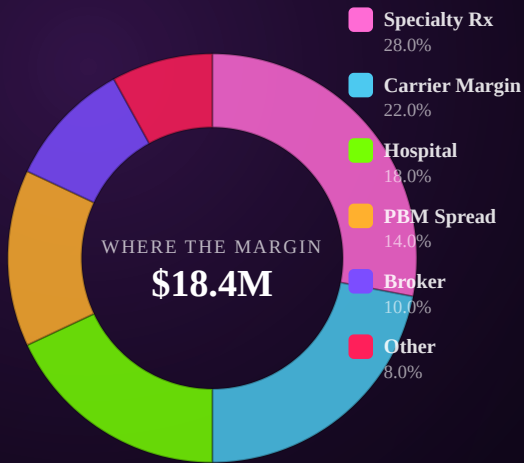
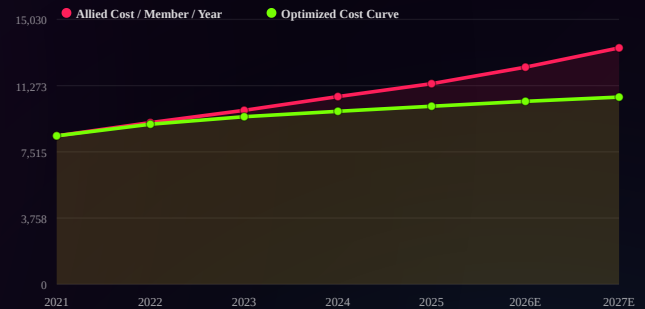


FIG. 32 · COST CURVE COMPRESSION POTENTIAL
PEPY trajectory · status quo extrapolation vs. McKinsey value-based care curve.



VALUE-BASED CARE

Bundled payments & capitation. McK research: 12–25% spend reduction with maintained outcomes.

CARE DELIVERY REDESIGN

Site-of-care · provider concentration · COE adoption. 8–14% net medical spend.

DIGITAL-FIRST OPS

Telehealth · digital therapeutics · self-service. 4–8% admin + access optimization.

MCK ALLIED IMPLICATION

Of Allied's \$18.4M annual spend, ~46% is captured by the specialty Rx and PBM spread layers. Reclaiming half of this via Tier 1 + Tier 2 levers represents **\$4.2M annual margin recovery** — independent of clinical trend.

Affordability Frontier.

PRODUCTION POSSIBILITY FRONTIER FOR HEALTHCARE
COST × OUTCOMES PARETO OPTIMIZATION

FIG. 33 · AFFORDABILITY × OUTCOMES FRONTIER
A=Current Allied · B=Tier 1 · C=Tier 1+2 · D=Full CEI · F=Theoretical frontier.
Movement upward-right = Pareto improvement.

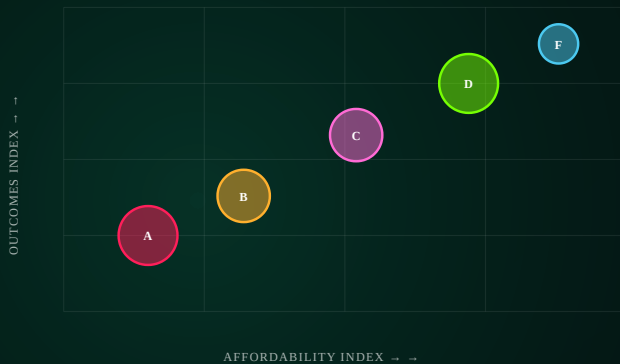
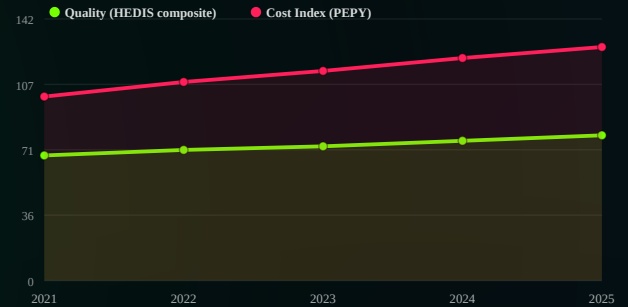


FIG. 34 · COST-OUTCOMES DECOUPLING

Allied cost outpacing quality gains. Both indices = 100 at 2021 baseline. Cost +27% · Quality +16%.



CURRENT POSITION (A)

Low affordability · low outcomes. Bottom-left quadrant. **Worst-of-both.**

PRACTICAL TARGET (C)

Tier 1+2 lever portfolio. Affordability +37 pts · outcomes +33 pts in 36 months.

FRONTIER TARGET (D)

Full Gen AI CEI · Kincaid IQ deterministic governance. Approaches theoretical frontier.

PARETO OPTIMALITY ANALYSIS

McK research demonstrates that **the affordability/outcomes frontier is NOT a fixed constraint** — it shifts outward with care delivery redesign, evidence-based plan architecture, and intelligent navigation. Allied's **38-point Pareto gap** represents the addressable opportunity space, none of which requires sacrificing outcomes.

Beyond Financials.

BCG TSI FRAMEWORK · STAKEHOLDER ECONOMICS
FINANCIAL + SOCIAL + ENVIRONMENTAL = TOTAL VALUE

FIG. 35 · TSI BUCKET CONTRIBUTION (15-YR PV \$M)

TSI is broader than EBITDA. **Family Stability** (\$6.8M) dominates total recoverable value.

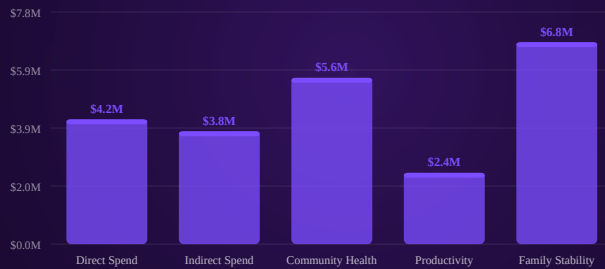


FIG. 36 · ESG / STAKEHOLDER MATURITY RADAR

Allied posture across six TSI dimensions. **Transparency & Community are critically underbuilt.**



DIRECT FINANCIAL

\$18.4M recoverable EBITDA. Standard accounting view.

INDIRECT ECONOMIC

Productivity gains from healthier workforce. Lower absenteeism, lower attrition.

SOCIAL DIVIDEND

Working family stability. Children's healthcare access. Community medical infrastructure support.

NOTRE DAME CLOSE · MORAL ECONOMICS

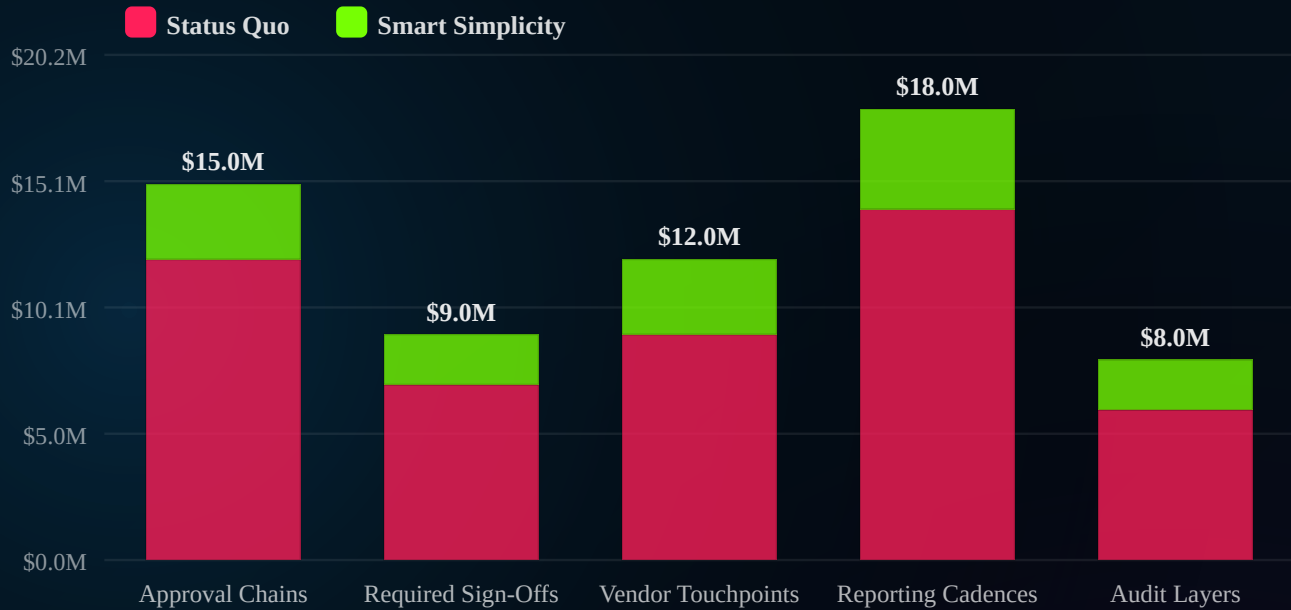
Behind every percentage point of healthcare trend is a working family deciding whether to fill a prescription, whether to keep a specialist appointment, whether to take a job they need but cannot afford to take. BCG TSI quantifies what every fiduciary already knows: **the value Allied creates extends beyond its own ledger**. The \$18.4M EBITDA defense is also \$6.8M of family stability over 15 years.

Operating Model Compression.

SIX SIMPLE RULES OF SMART SIMPLICITY®
REDUCE COMPLICATEDNESS · MULTIPLY AUTONOMY

FIG. 37 · DECISION COMPLEXITY · STATUS QUO VS. SMART SIMPLICITY

Allied benefits committee operating-model complexity vs. BCG SS target state. **~70% reduction in approval chains; ~67% reduction in vendor touchpoints.**



RULE 1

Understand context

What each actor (CFO, CHRO, Treasurer) actually needs.

RULE 2

Reinforce integrators

Treasurer + Plan Sponsor as natural integrators across silos.

RULE 3

Increase total quantity of power

Tier 0 substrate empowers every actor with cleaner data.

RULE 4

Increase reciprocity

Vendors compensated on outcomes, not procurement volume.

RULE 5

Extend shadow of the future

Multi-year incentive alignment with vendors. No annual extraction.

RULE 6

Reward cooperators

Vendor scorecard rewards reduce friction, not optimize for narrow KPI.

SMART SIMPLICITY · ALLIED COMPRESSION TARGET

Compress decision complexity by 67%; multiply autonomy of plan sponsor + treasurer + actuary integrator trio. **Capacity unlocked = additional 0.3 FTE of strategic bandwidth**, redirected to Tier 2 strategic execution. The operating model itself becomes a Tier 0 lever.

Disruptor Landscape.

OW HEALTH MARKET DISRUPTORS
VERTICAL INTEGRATION · CONSUMER CARE

FIG. 38 · COST TRAJECTORY · TRADITIONAL VS. DISRUPTOR

5-year indexed cost trajectory. Disruptor pathway flattens the curve through vertical integration efficiencies.

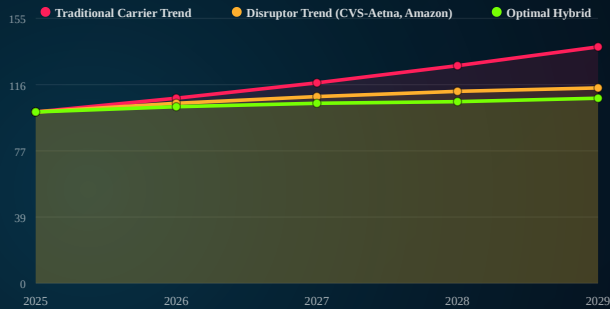
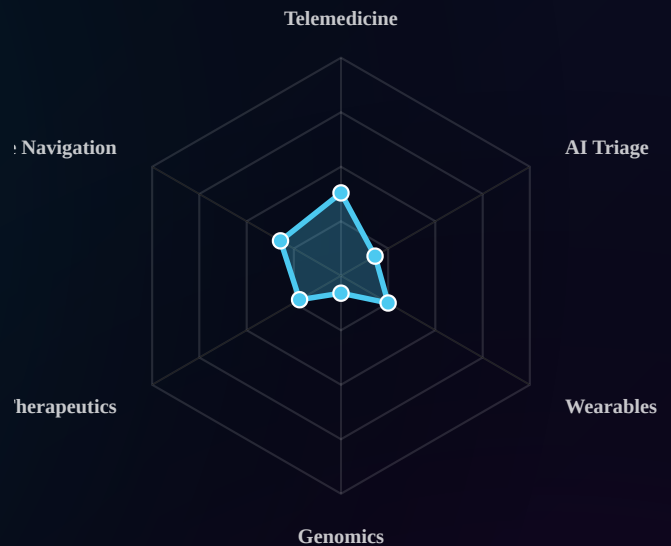


FIG. 39 · INNOVATION ADOPTION MATURITY

Allied posture across health innovation dimensions. AI triage and genomics are **critically underbuilt**.



CVS HEALTH-AETNA

PBM + payer + retail clinic + Signify. Vertically integrated cost curve compression.

UHG-OPTUM

Payer + PBM + provider + tech. Largest vertical play. Allied bias check required.

AMAZON HEALTH

PillPack · One Medical · Pharmacy. Consumer-direct disruption from outside the system.

DISRUPTOR STRATEGY · ALLIED PLAY

Allied does NOT need to vertically integrate. Allied needs to **capture the cost-curve benefits of vertical integration without surrendering the governance benefits of independence**. Direct contracting (L20), reference pricing (L21), and onsite clinics (L22) give Allied disruptor-grade economics with sovereign governance.

Risk-Based Capital Adequacy.

RBC METHODOLOGY · NAIC ANALOG
STOP-LOSS ADEQUACY · TAIL CAPITAL

FIG. 40 · TAIL CAPITAL REQUIREMENT · INDUSTRY DISTRIBUTION

CVaR 99% capital reserve. Allied sits at industry Q90+ — driven by Tier 0/1 substrate gaps.

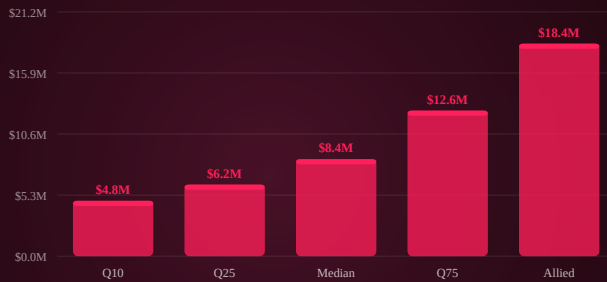


FIG. 41 · CAPITAL ADEQUACY RATIO

RBC adequacy = available capital / required capital. **58 / 100** — below large-plan benchmark.



Target: ≥ 85. Gap closeable through stop-loss re-underwrite + Master Trust optimization.

CATASTROPHIC SHOCK CAPITAL

High-cost claimant tail · gene therapy · NICU.
Stop-loss attachment optimization required.

TREND VOLATILITY CAPITAL

σ/μ ratio capital buffer. Falls with credibility
increases and predictive maturity.

OPERATIONAL CAPITAL

Vendor failure · regulatory event · cybersecurity.
Tier 4 governance reduces requirement.

RBC RECOMMENDATION FOR ALLIED

Adopt NAIC-analog RBC discipline despite ERISA exemption. **Voluntary capital adequacy reporting is the strongest possible Tibble-proof posture.** Quantify capital reserves against CVaR 99% and report quarterly. Capital adequacy ratio target ≥ 85 within 24 months.

Twin Operating-Model Lenses.

KPMG 8 CAPABILITIES · PwC 5 ELEMENTS
TRUSTED + DIGITAL + OUTCOME-ORIENTED

FIG. 42 · KPMG CONNECTED ENTERPRISE · 8 CAPABILITIES

Allied posture across KPMG's 8 customer-centric capabilities. All 8 sub-50; ecosystem and partner integration critically below.

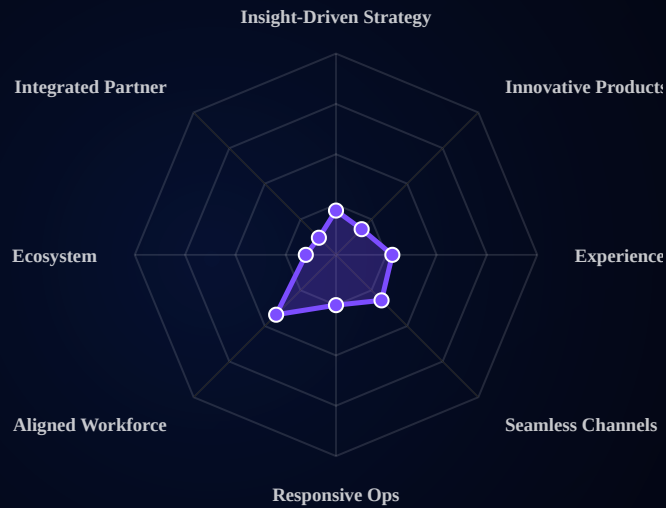
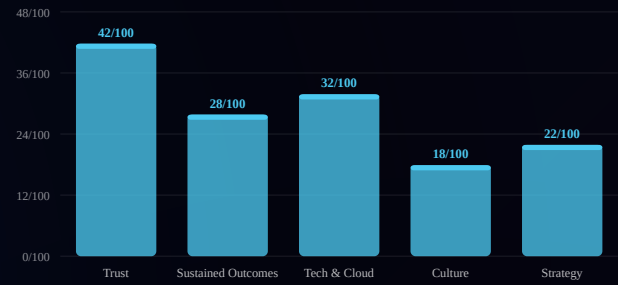


FIG. 43 · PwC NEW EQUATION · 5 ELEMENTS

Allied posture across PwC's "New Equation". Trust is the relative strength; sustained outcomes are the weak point.



TRUSTED

FSI + transparency = trust capital.
PwC primary axis.

DIGITAL

Kincaid IQ as substrate. KPMG
digital ecosystem enabler.

OUTCOME-ORIENTED

Vendor contracts paid on outcomes.
Both frameworks converge.

PARTNER-INTEGRATED

Brown & Brown · SiriusB iQ as
ecosystem partners.

CROSS-FRAMEWORK SYNTHESIS

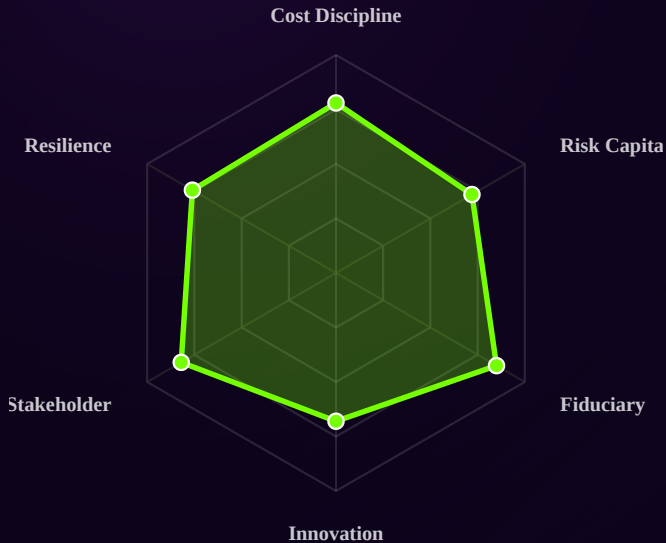
KPMG and PwC converge on a common conclusion: in 2026 and beyond, **trusted + digital + outcome-oriented + partner-integrated** is no longer aspirational. It is the minimum viable architecture for capital-intensive benefits programs. Allied's gap is not in any single dimension; **it is in the integration across all four.**

CROSS-FIRM SYNTHESIS

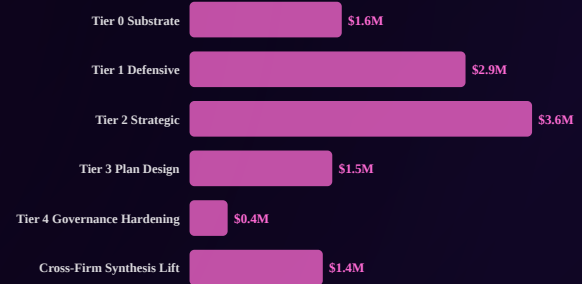
11 Firms · 1 Conclusion.

McK · BAIN · BCG · EY-P · DELOITTE · OW · CROWE
KPMG · PwC · ACCENTURE · GS LIABILITY DISCIPLINE**FIG. 44 · POST-EXECUTION POSTURE · TARGET STATE**

Allied target-state posture across all 6 cross-firm convergence dimensions. All targets ≥ 68 . **Fiduciary highest priority axis.**

**FIG. 45 · CONSOLIDATED RECOMMENDATION IMPACT (\$M/YR)**

Annualized EBITDA recovery by lever tier, with \$1.4M attributable to cross-firm synthesis benefits beyond any single framework.



CONVERGENCE TABLE · WHERE ALL ELEVEN FIRMS AGREE

CONVERGENCE POINT	MCK	BAIN	BCG	EY-P	DELOITTE	OW	CROWE	KPMG	PWC
Benefits = unmeasured capital allocation	✓	✓	✓	★	✓	✓	✓	✓	✓
Vertical integration captures margin	★	✓	✓	✓	✓	★	✓	✓	✓
Outcomes-based vendor contracting required	✓	★	★	✓	✓	✓	✓	✓	★
Care delivery redesign > price negotiation	★	✓	✓	✓	★	✓	✓	✓	✓
Fiduciary discipline is the moat	✓	✓	✓	✓	✓	✓	★	✓	✓

★ = framework's signature claim · ✓ = framework agrees in substance

FINAL CROSS-FIRM SYNTHESIS

Eleven of the world's most respected advisory and audit firms — across strategy, transactions, risk, and operating model — converge on a single conclusion for Allied Solutions LLC. The annual outflow of **\$18.4M** is not a healthcare expense; it is a capital allocation decision. The **\$11.4M annualized recovery envelope** sits within both the practical and the philosophical capability of the organization. The fiduciary mandate under ERISA §404(a), the executive mandate under enterprise capital discipline, and the moral mandate to working families all point in the same direction. **The case is closed. The path is engineered. The decision is the board's.**

PRINCIPAL ANALYST

Jeremiah F. Shrack · Chair, CEO & Founder · Kincaid Risk Management Co. · SiriusB IQ

EVIDENCE SPINE

No Anchor, No Claim. No Lineage, No Publish.